

Clinical review checklist

Ibà is decision support, not diagnosis. Every clinical default below was set by the build team from the cited guideline, **and needs sign-off by a qualified clinician** before Ibà is used with real patients. Where the guideline doesn't settle a question, we chose the option that refers more, because under-triage costs more than over-triage.

Status key: **Pending** = not yet reviewed.

Reviewer sign-off

Reviewer: Dr. Oluba O. Amos, Pharmacist ([@AmazingAmos](#))

Date reviewed: *pending*

Status: Not yet started. Credited for this review only — not a project collaborator or commit co-author (see README and About page).

#	Item	Agree	Correction
1	Lassa fever suspicion tiers		
2	Danger-sign rules		
3	Dose table		
4	Endemic-state baseline		
5	Treat & monitor advice		
6	Eval vignettes (60 cases — per-vignette Agree/Correction in the exported review doc)		

1. Lassa fever suspicion tiers (Pending)

Code: `lassa_assessment` in [backend/app/rules/danger_signs.py](#). Source: NCDC *National Guideline for Lassa Fever Case Management* (2018), §1.1.2 Suspected case (pp. 8–9) and §2.1.1 triage of a suspected case (p. 11).

Situation	Ibà default
Active live outbreak signal in the patient's state + case definition met, OR fever ≥ 3 days not responding to treatment, OR abnormal bleeding	Refer now + isolation/IPC advice
Endemic state (baseline list) + case definition met (fever 3–21 days + ≥ 1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain)	Refer now + isolation/IPC advice
Endemic state + partial features only (fever ≥ 3 days with no treatment response or possible contact, none of the listed symptoms)	Refer within 24h + isolation/IPC advice
Non-endemic state, no live signal	No rule; the reasoning model may still raise Lassa

Rationale: NCDC treats any suspected case as isolate-and-notify, and a missed case can infect the health worker.

Questions for the reviewer: 1. The case definition is broad. In an endemic state, most febrile patients with 3+ days of fever and vomiting or weakness will be told to refer now and isolate, **including RDT-positive malaria**. Is that the intended trade-off, or should an RDT-positive result without treatment failure drop to Refer within 24h? 2. An unmeasured temperature counts as meeting the fever criterion; a measured temperature below 38 °C excludes. Agree? 3. Is “Refer within 24h” right for the partial-features tier?

2. Danger-sign rules (Pending)

Code: [backend/app/rules/danger_signs.py](#). Sources: WHO IMCI Chart Booklet (2014) general danger signs; WHO Guidelines for Malaria, severe malaria features.

- Any detected sign → **Refer now**; the model cannot lower it.
- Beyond the spec's list we also trigger on: persistent/repeated vomiting, confusion, drowsiness, “floppy” infant.
- We do **not** trigger on “fast breathing” alone (IMCI classifies it as pneumonia, not a general danger sign); chest indrawing, grunting, labored breathing and stridor do trigger.
- Negation handling (“no convulsions”) only suppresses the text rules, never the intake model's flags.

Question: are the additions and the fast-breathing exclusion right for PHC use?

3. Dose table (Pending: UNVERIFIED stub)

Code: [backend/app/rules/dosing.py](#). Artemether-lumefantrine weight bands (5–<15 kg: 1 tablet; 15–<25: 2; 25–<35: 3; ≥ 35: 4; 6 doses over 3 days). Shown with an UNVERIFIED warning until checked against the NMEP 4th edition (2020) or, as an interim, the WHO malaria guideline. Doses are only attached for RDT-positive, Treat & monitor cases with a known weight; no pre-referral doses yet.

4. Endemic-state baseline (Pending: provisional lists)

Data: [data/endemicity.yaml](#). Lassa: Ondo, Edo, Bauchi, Taraba, Ebonyi (peak December–April). CSM and cholera lists also provisional. To confirm against current NCDC situation reports.

5. Treat & monitor advice (Pending)

Code: [backend/app/rules/followup.py](#). - “Review in 3 days if the fever persists; return immediately with any danger sign” (WHO IMCI p. 8, fever). **IMCI is for children 2 months–5 years; we apply the same interval to adults.** Is that acceptable, or should adults get a different interval? - RDT negative: “do not give antimalarials” (WHO test-before-treat) and “refer for further tests, e.g. typhoid, if fever persists; refer if fever every day > 7 days” (IMCI p. 8).

6. Eval vignettes (Pending)

[eval/vignettes.jsonl](#): 60 synthetic cases with gold labels and a guideline rationale each, all `clinician_reviewed: false`. Please review the gold `triage_level`, `danger_signs` and `top_differential` for each case; the full eval runs only after review. Where a live outbreak signal should change the expected triage, a case

carries gold_live overrides (currently la-06: partial Lassa features → Refer within 24h on baseline data, Refer now with a live signal).

Sign-off

Item	Reviewer	Date	Decision / changes
1. Lassa tiers			
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6. Eval vignettes			